

Psychiatric Pre-Screening

Framework, Question Set and Scoring Guide

Version	1.0	Date	June 2025
Target Users	Adults 18+ (all ages)	Primary Focus	Adults 50+ but applicable to all
Conditions Covered	10 psychiatric conditions	Total Questions	64 questions across 10 sections
Clinical Basis	PHQ-9, GAD-7, MDQ, PCL-5, OCI-R, PRIME Screen, AUDIT, ASRS, SCOFF, C-SSRS		
Scoring Model	Weighted domain scoring + red flag override + age multiplier		
Document Purpose	App design, questionnaire planning, clinical validation reference		

Clinical Disclaimer: This document is a pre-screening triage tool only. It cannot diagnose, confirm, or rule out any psychiatric or medical condition. Questions and scoring weights are derived from publicly available, validated clinical instruments. The composite scoring model described here has not been independently validated in a clinical trial. A high score does not mean a diagnosis. A low score does not rule one out. Independent clinical validation and ethics review are strongly recommended before any public deployment. Only a qualified mental health professional can perform a full assessment and provide a diagnosis.

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01 App Flow and Adaptive Logic

The app does not show all 64 questions to every user. It uses adaptive logic to branch into relevant sections based on the user's answers. Most users will see 15 to 25 questions. This keeps the experience under 8 minutes and avoids overwhelming people who are already distressed.

Step 1

WELCOME and CONSENT

User reads a plain-language explanation of what the tool does and what it cannot do. They confirm they understand this is not a diagnosis. Informed consent is recorded.

Step 2

AGE GATE

User selects their age bracket: Under 50 / 50 to 64 / 65 to 74 / 75 and above. This sets the Age Multiplier used in the scoring formula later.

Step 3

CORE SECTIONS (shown to everyone)

Section A: Depression (9 questions) and Section B: Anxiety (7 questions) are shown to all users. These are the two most common conditions and act as the entry point for branching.

Step 4

ADAPTIVE BRANCHING (shown based on answers)

Based on scores in Sections A and B, the app decides which additional sections to show: - BIP1 or BIP2 score 2+: show Section C (Bipolar) - PTS1 or PTS3 score 2+: show Section D (PTSD) - OCD1 or OCD5 score 2+: show Section E (OCD) - PSY1 or PSY2 score 1+: show Section F (Psychosis) immediately and flag urgent - ALC1 score 2+: show Section G (Alcohol) - ADH2 or ADH4 score 2+: show Section H (ADHD) - EAT1 or EAT5 score 1+: show Section I (Eating Disorders) - DEP9 score 1+: show Section J (Suicide Risk) and show crisis resources

Step 5

RED FLAG CHECK (runs in real time)

The app checks red flag questions as each answer is submitted, not at the end. If a red flag triggers, the app responds immediately with an appropriate message before the user continues.

Step 6

SCORED RESULTS

The app shows a domain breakdown bar chart, the overall result tier (Green / Yellow / Orange / Red), and a plain-language recommendation. No medical diagnosis is stated.

**Step
7****NEXT STEPS**

GP or psychiatrist referral language, crisis resources if applicable, option to download or share the report, and a reminder that results should be shown to a health professional.

02 Global Scoring Model

Step	Action	Formula / Detail
1	Add raw scores in each domain	Sum all answer values (0 to 3) for each question in the domain. For questions with 1.5x weight, multiply the answer value by 1.5 before adding.
2	Normalise each domain score	$\text{domain_normalised} = (\text{domain_raw_score} / \text{domain_max_raw_score}) \times 100$
3	Apply the domain weight	$\text{domain_weighted} = \text{domain_normalised} \times (\text{domain_weight_percentage} / 100)$
4	Calculate the Base Score	$\text{base_score} = \text{sum of all domain_weighted scores}$. This gives a value between 0 and 100.
5	Apply the Age Multiplier	Under 50: x 0.7 50 to 64: x 1.0 65 to 74: x 1.2 75 and above: x 1.4
6	Calculate the Final Score	$\text{final_score} = \text{minimum of } (\text{base_score} \times \text{age_multiplier}) \text{ and } 100$. The score is capped at 100.
7	Run the Red Flag Override	If any red flag question scores 2 or 3, the result is automatically raised to at least Moderate Concern regardless of the total score. Some red flags (Section J) trigger immediate crisis resources before any score is calculated.

03 Answer Scale Reference

All questions use this 4-point scale unless a special scale is noted in the section header.

Answer	Label	Score	What it means
Option A	Not at all	0	This has not happened to me
Option B	Several days	1	Happens a few days a month
Option C	More than half the days	2	Happens most days of the week
Option D	Nearly every day	3	Happening almost every day

04 Result Tiers

The final score places the user in one of four tiers. Red flag overrides can raise the tier regardless of score.

Score Range	Tier	Colour	Recommended Action
0 to 20	Low Concern	Green	No significant signs. Stay aware, recheck in 12 months.

21 to 45	Mild Concern	Yellow	Some signs present. Mention at next routine GP or psychiatrist visit.
46 to 65	Moderate Concern	Orange	Multiple signs across domains. Book an appointment this month.
66 to 100	High Concern	Red	Strong signs present. See a mental health professional soon.

Section A Depression

Based on PHQ-9 | Kroenke et al.,
JGIM 2001

Screens for depressive disorder. Most widely used depression tool in clinical practice worldwide.

What it screens	Depression is one of the most common mental health conditions. It involves persistent low mood, loss of interest, and physical symptoms like sleep and appetite changes. The PHQ-9 is the gold standard self-report tool for depression and is used in GP clinics, hospitals, and research worldwide. A score of 10 or above on the original PHQ-9 suggests moderate depression and warrants clinical review.
Scoring note	Standard 4-point scale (0 to 3). Total possible: 27. PHQ-9 original cutoffs: 5 mild, 10 moderate, 15 severe, 20 very severe.

Domain Weights

Domain	Questions	Weight	Max Score
Core Depressive Symptoms	DEP1, DEP2, DEP6, DEP8	40%	12
Neurovegetative Symptoms (Sleep, Energy, Appetite)	DEP3, DEP4, DEP5	35%	9
Cognitive and Concentration	DEP7	15%	3
Suicidal Ideation	DEP9	10%	3 — Red Flag

Questions

For each question, tick the box that best describes how often you have experienced this over the past 2 weeks. RF column marks red flag questions.

#	Question	Not at all (0)	Several days (1)	More than half days (2)	Nearly every day (3)	RF
DEP1	I have felt little interest or pleasure in doing things I normally enjoy.	☐	☐	☐	☐	
DEP2	I have felt down, hopeless, or empty inside.	☐	☐	☐	☐	
DEP3	I have had trouble falling asleep, staying asleep, or I sleep too much.	☐	☐	☐	☐	
DEP4	I feel tired or have very little energy, even without much effort.	☐	☐	☐	☐	
DEP5	My appetite has changed a lot. I eat too much or too little.	☐	☐	☐	☐	
DEP6	I feel bad about myself. I feel like a failure or like I have let people down.	☐	☐	☐	☐	

DEP7	I find it hard to concentrate on things like reading, watching TV, or work.	☐	☐	☐	☐	
DEP8	I move or speak so slowly that others have noticed, or I feel very restless and fidgety.	☐	☐	☐	☐	
DEP9	I have had thoughts that I would be better off dead, or thoughts of hurting myself.	☐	☐	☐	☐	!

Red Flag Rule: DEP9 is a mandatory red flag. Any score of 1 or above on this question triggers an immediate referral message and crisis resources, regardless of the total score. Do not wait for the section to be completed.

References

[1] Kroenke K, Spitzer RL, Williams JB. The PHQ-9: validity of a brief depression severity measure. J Gen Intern Med. 2001;16(9):606-613. PMID: 11556941

[2] Spitzer RL, et al. Validation and utility of a self-report version of PRIME-MD: the PHQ primary care study. JAMA. 1999;282(18):1737-1744. PMID: 10568646

Section B Generalised Anxiety

Based on GAD-7 | Spitzer et al., Arch Intern Med 2006

Screens for generalised anxiety disorder. Routinely paired with PHQ-9 in clinical intake assessments.

What it screens

Anxiety is characterised by excessive, uncontrollable worry about everyday events. The GAD-7 is a 7-item self-report scale validated for detecting generalised anxiety disorder. It is the most widely used anxiety screening tool in primary care and psychiatry globally. A score of 10 or above suggests moderate anxiety and warrants further assessment.

Scoring note

Standard 4-point scale (0 to 3). Total possible: 21. GAD-7 cutoffs: 5 mild, 10 moderate, 15 severe.

Domain Weights

Domain	Questions	Weight	Max Score
Core Anxiety and Worry	ANX1, ANX2, ANX3	45%	9
Physical and Behavioural Anxiety	ANX4, ANX5	30%	6
Irritability and Anticipatory Fear	ANX6, ANX7	25%	6

Questions

For each question, tick the box that best describes how often you have experienced this over the past 2 weeks. RF column marks red flag questions.

#	Question	Not at all (0)	Several days (1)	More than half days (2)	Nearly every day (3)	RF
ANX 1	I have felt nervous, anxious, or on edge.	☐	☐	☐	☐	
ANX 2	I have not been able to stop or control my worrying.	☐	☐	☐	☐	
ANX 3	I worry too much about lots of different things at the same time.	☐	☐	☐	☐	

ANX 4	I find it hard to relax, even when I have free time.	☐	☐	☐	☐	
ANX 5	I feel so restless that it is hard to sit still.	☐	☐	☐	☐	
ANX 6	I get easily annoyed or irritated by small things.	☐	☐	☐	☐	
ANX 7	I feel something terrible is going to happen, even when things seem fine.	☐	☐	☐	☐	

No automatic red flag override for this section. Use total score and result tiers only.

References

[1] Spitzer RL, et al. A brief measure for assessing generalized anxiety disorder: the GAD-7. Arch Intern Med. 2006;166(10):1092-1097. PMID: 16717171

[2] Lowe B, et al. Validation and standardization of the Generalized Anxiety Disorder Screener (GAD-7) in the general population. Med Care. 2008;46(3):266-274. PMID: 18388841

Section C Bipolar Disorder

Based on MDQ | Hirschfeld et al., Am J Psychiatry 2000

Screens for bipolar I and II disorder. Focuses on periods of elevated mood, energy, and impulsive behaviour.

What it screens	Bipolar disorder involves episodes of abnormally elevated or irritable mood (mania or hypomania) alternating with episodes of depression. The Mood Disorder Questionnaire (MDQ) was developed to help identify people who may have undiagnosed bipolar disorder. It asks specifically about periods where multiple high-energy symptoms occurred together. A positive MDQ screen requires 7 or more yes responses AND functional impairment AND symptom clustering.
Scoring note	Adapted to 4-point scale (0 to 3) for consistency. Total possible: 21. Pay special attention to symptom clustering: if 3 or more questions score 2 or above in the same time period, escalate regardless of total.

Domain Weights

Domain	Questions	Weight	Max Score
Elevated Mood and Energy Episodes	BIP1, BIP2, BIP3, BIP4	50%	12
Impulsive and Risk-Taking Behaviour	BIP5, BIP6	30%	6 — BIP6 is Red Flag
Mood Cycling and Instability	BIP7	20%	3

Questions

For each question, tick the box that best describes how often you have experienced this over the past 2 weeks. RF column marks red flag questions.

#	Question	Not at all (0)	Several days (1)	More than half days (2)	Nearly every day (3)	RF
BIP1	I have had periods where I felt unusually high, elated, or on top of the world. Not just a good mood but noticeably different from normal.	☐	☐	☐	☐	

BIP2	I have had periods where I needed much less sleep than usual but still felt full of energy the next day.	☐	☐	☐	☐	
BIP3	I have been much more talkative than usual, or felt pressure to keep talking and could not stop.	☐	☐	☐	☐	
BIP4	My thoughts raced so fast that it was hard to keep up with them or slow them down.	☐	☐	☐	☐	
BIP5	I was so active or tried to do so many things at once that I got into trouble because of it.	☐	☐	☐	☐	
BIP6	I was more interested in sex than usual, or made decisions about money, business, or relationships that I later regretted because they were risky or impulsive.	☐	☐	☐	☐	!
BIP7	My mood switches noticeably between feeling very high and feeling very low, sometimes within the same week or even the same day.	☐	☐	☐	☐	

Red Flag Rule: BIP6 covers impulsive and risky behaviour that can cause serious financial, legal, or personal harm. A score of 2 or 3 escalates to at least Moderate Concern. This question should also prompt a note to the clinician that the patient may have unrecognised bipolar disorder.

References

- [1] Hirschfeld RM, et al. Development and validation of a screening instrument for bipolar spectrum disorder: the Mood Disorder Questionnaire. Am J Psychiatry. 2000;157(11):1873-1875. PMID: 11058490
- [2] Miller CJ, et al. Sensitivity and specificity of the Mood Disorder Questionnaire for detecting bipolar disorder. J Affect Disord. 2004;81(2):167-171. PMID: 15306143

Section D PTSD

Based on PCL-5 | Weathers et al.,
VA National Center for PTSD 2013

Screens for post-traumatic stress disorder aligned to DSM-5 criteria. Covers intrusion, avoidance, mood, and hyperarousal.

What it screens

PTSD occurs after experiencing or witnessing a traumatic event. Symptoms include reliving the event through flashbacks or nightmares, avoiding reminders, feeling emotionally numb, and being on constant alert. The PCL-5 is the standard PTSD screening tool, aligned to the DSM-5 diagnostic criteria. A total score of 33 or above on the full 20-item PCL-5 is a provisional PTSD diagnosis. This 8-item adapted version focuses on the most clinically significant items.

Scoring note

Standard 4-point scale (0 to 3). Total possible: 24. A score of 14 or above on this 8-item version suggests significant PTSD symptoms.

Domain Weights

Domain	Questions	Weight	Max Score
Intrusion Symptoms (Re-experiencing)	PTS1, PTS2, PTS3, PTS4	40%	12
Avoidance and Emotional Numbing	PTS5, PTS6	30%	6

Hyperarousal and Reactive Symptoms	PTS7, PTS8	30%	6 — PTS8 is Red Flag
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Questions

For each question, tick the box that best describes how often you have experienced this over the past 2 weeks. RF column marks red flag questions.

#	Question	Not at all (0)	Several days (1)	More than half days (2)	Nearly every day (3)	RF
PTS1	I have had repeated, upsetting memories, thoughts, or images of a stressful experience from the past.	☐	☐	☐	☐	
PTS2	I have had repeated bad dreams or nightmares related to a stressful past event.	☐	☐	☐	☐	
PTS3	I have suddenly felt or acted as if a stressful past event was happening again right now. This is also called a flashback.	☐	☐	☐	☐	
PTS4	I feel very upset when something reminds me of a stressful past experience.	☐	☐	☐	☐	
PTS5	I avoid thinking about, talking about, or doing things that remind me of a stressful past experience.	☐	☐	☐	☐	
PTS6	I feel emotionally cut off or distant from people around me, even people I care about.	☐	☐	☐	☐	
PTS7	I am constantly on guard, watchful, or easily startled, even when I am in a safe place.	☐	☐	☐	☐	
PTS8	I have had angry outbursts or acted in a way that surprised or scared people around me.	☐	☐	☐	☐	!

Red Flag Rule: PTS8 (angry outbursts or frightening behaviour) is a safety concern for both the patient and those around them. A score of 2 or 3 escalates to at least Moderate Concern and prompts a safety note in the report.

References

- [1] Weathers FW, et al. The PTSD Checklist for DSM-5 (PCL-5). National Center for PTSD, VA. 2013. Available at: www.ptsd.va.gov
- [2] Blevins CA, et al. The Posttraumatic Stress Disorder Checklist for DSM-5 (PCL-5): development and initial psychometric evaluation. J Trauma Stress. 2015;28(6):489-498. PMID: 26606250

Section E OCD

Based on OCI-R | Foa et al., Psychol Assessment 2002

Screens for obsessive-compulsive disorder across 6 symptom subtypes including washing, checking, ordering, hoarding, and intrusive thoughts.

What it screens

OCD involves unwanted repetitive thoughts (obsessions) and behaviours or mental acts done to reduce anxiety (compulsions). The Obsessive-Compulsive Inventory Revised (OCI-R) is an 18-item self-report measure. This 6-item adapted version covers the main subtypes. A score of 21 or above on the full OCI-R is the standard clinical cutoff. OCD affects around 2% of the population and is often misdiagnosed or undetected for years.

Scoring note

Standard 4-point scale (0 to 3). Total possible: 18. Each question covers a different OCD subtype. A score of 10 or above on this 6-item version suggests clinically significant OCD symptoms.

Domain Weights

Domain	Questions	Weight	Max Score
Washing and Contamination	OCD1	17%	3
Checking	OCD2	17%	3
Ordering and Symmetry	OCD3	17%	3
Hoarding	OCD4	17%	3
Intrusive Thoughts (Obsessing)	OCD5	16%	3
Neutralising and Rituals	OCD6	16%	3

Questions

For each question, tick the box that best describes how often you have experienced this over the past 2 weeks. RF column marks red flag questions.

#	Question	Not at all (0)	Several days (1)	More than half days (2)	Nearly every day (3)	RF
OCD 1	I wash or clean things much more than is necessary. For example my hands, the house, or objects around me.	☐	☐	☐	☐	
OCD 2	I check things over and over again, like locks, switches, the cooker, or whether I sent a message correctly.	☐	☐	☐	☐	
OCD 3	I feel very upset if things around me are not in a certain order or arranged in exactly the right way.	☐	☐	☐	☐	
OCD 4	I feel the urge to collect or keep things that most people would throw away without thinking.	☐	☐	☐	☐	
OCD 5	I have unwanted thoughts that feel stuck in my head, even when I try hard to push them out or ignore them.	☐	☐	☐	☐	
OCD 6	I repeat certain actions, words, or routines to stop something bad from happening, even though I know it probably does not make logical sense.	☐	☐	☐	☐	

No automatic red flag override for this section. Use total score and result tiers only.

References

[1] Foa EB, et al. The Obsessive-Compulsive Inventory: development and validation of a short version. Psychol Assess. 2002;14(4):485-496. PMID: 12501574

[2] Abramowitz JS, Tolin DF, Diefenbach GJ. Measuring change in OCD: sensitivity of the OCI-R. J Psychopathol Behav Assess. 2005;27:317-324. DOI: 10.1007/s10862-005-2411-y

Section F Psychosis and Early Psychosis

Based on PRIME Screen | Miller et al., Schizophr Bull 2003

Screens for psychotic experiences including hallucinations, delusions, and thought disorder. All 5 core questions are red flags.

What it screens

Psychosis involves a break from reality, most commonly through hallucinations (hearing or seeing things that others cannot) and delusions (strongly held false beliefs). The PRIME Screen is a self-report tool developed to detect prodromal and early psychosis. Schizophrenia affects about 1% of the population. Early detection dramatically improves outcomes. This section requires careful handling as even one positive response warrants clinical follow-up.

Scoring note

Standard 4-point scale (0 to 3). Total possible: 18. Unlike other sections, even a single question scored 2 or 3 is a High Concern trigger. Threshold for referral is low because early intervention is critical.

Domain Weights

Domain	Questions	Weight	Max Score
Hallucinations (Auditory and Visual)	PSY1, PSY2	35%	6 — Both Red Flags
Paranoia and Persecutory Beliefs	PSY3	20%	3 — Red Flag
Thought Insertion and Passivity	PSY4	20%	3 — Red Flag
Thought Disorganisation	PSY5	10%	3
Fixed False Beliefs	PSY6	15%	3 — Red Flag

Questions

For each question, tick the box that best describes how often you have experienced this over the past 2 weeks. RF column marks red flag questions.

#	Question	Not at all (0)	Several days (1)	More than half days (2)	Nearly every day (3)	RF
PSY1	I have heard sounds, noises, or voices that other people around me could not hear.	☐	☐	☐	☐	!
PSY2	I have seen things, shadows, or figures that other people around me could not see.	☐	☐	☐	☐	!
PSY3	I have had a strong feeling that people are watching me, following me, or trying to harm me, without a clear reason for this belief.	☐	☐	☐	☐	!
PSY4	I have had the experience that thoughts were being put into my head by someone else, or that my own thoughts were being taken away or broadcast out loud.	☐	☐	☐	☐	!
PSY5	My thoughts have felt so mixed up, jumbled, or fast that I could not follow them or explain them clearly to other people.	☐	☐	☐	☐	
PSY6	I have believed something very strongly that people close to me said was clearly not true, but I still felt certain it was real and they were wrong.	☐	☐	☐	☐	!

Red Flag Rule: PSY1, PSY2, PSY3, PSY4, and PSY6 are all red flags. Any score of 2 or 3 on any single one of these questions automatically escalates the result to High Concern and triggers urgent referral messaging. In a clinical app, this section should always include a note that results will be shared with a health professional.

References

[1] Miller TJ, et al. Prodromal Assessment with the Structured Interview for Prodromal Syndromes and the Scale of Prodromal Symptoms: predictive validity, inter-rater reliability and training to reliability. Schizophr Bull. 2003;29(4):703-715. PMID: 14989408

[2] Heinimaa M, et al. The PROD-screen: a screen for prodromal symptoms of psychosis. Int J Methods Psychiatr Res. 2003;12(2):92-104. DOI: 10.1002/mpr.146

Section G Alcohol Use

Based on AUDIT and AUDIT-C |
Saunders et al., Addiction 1993

Screens for hazardous, harmful, and dependent alcohol use. Uses a mixed scoring scale for the first three questions.

What it screens

The AUDIT (Alcohol Use Disorders Identification Test) was developed by the World Health Organisation and is the most validated alcohol screening tool in the world. It is used in hospitals, GP clinics, and emergency settings across more than 50 countries. AUDIT-C (the first 3 questions) is a shorter version used for rapid screening. Questions ALC1 to ALC3 use a different scoring scale specific to the AUDIT. ALC4 and ALC5 use the standard 4-point scale.

Scoring note

MIXED SCALE. ALC1, ALC2, ALC3 use the AUDIT-specific options shown below. ALC4 and ALC5 use the standard 0 to 3 scale. Total AUDIT-C score of 4+ (men) or 3+ (women) is a positive screen.

Note on ALC1, ALC2, ALC3: These three questions use the original AUDIT answer options with scores from 0 to 4. The answer options are printed inside the question text below. ALC4 and ALC5 use the standard 0 to 3 scale.

Domain Weights

Domain	Questions	Weight	Max Score
Frequency and Quantity of Use	ALC1, ALC2	30%	8
Binge Drinking Episodes	ALC3	30%	4 — Red Flag
Control and Dependence	ALC4	20%	3
Social and Functional Harm	ALC5	20%	3 — Red Flag

Questions

For each question, tick the box that best describes how often you have experienced this over the past 2 weeks. RF column marks red flag questions.

#	Question	Not at all (0)	Several days (1)	More than half days (2)	Nearly every day (3)	RF
ALC1	How often do you have a drink containing alcohol? A: Never (0) B: Monthly or less (1) C: 2 to 4 times a month (2) D: 2 to 3 times a week (3) E: 4 or more times a week (4)	☐	☐	☐	☐	
ALC2	On a typical drinking day, how many standard drinks do you have? A: 1 or 2 (0) B: 3 or 4 (1) C: 5 or 6 (2) D: 7 to 9 (3) E: 10 or more (4)	☐	☐	☐	☐	

ALC3	How often do you have 6 or more drinks on a single occasion? A: Never (0) B: Less than monthly (1) C: Monthly (2) D: Weekly (3) E: Daily or almost daily (4)	☐	☐	☐	☐	!
ALC4	I have felt that I should cut down my drinking but found it difficult to do so.	☐	☐	☐	☐	
ALC5	My drinking has caused problems at work, at home, or in my relationships with other people.	☐	☐	☐	☐	!

Red Flag Rule: ALC3 (binge drinking frequency) and ALC5 (drinking causing real-world harm) are red flags. A score of 2 or more on either question escalates to at least Moderate Concern. These two questions together with ALC4 closely mirror the ICD-11 criteria for harmful use of alcohol.

References

[1] Saunders JB, et al. Development of the Alcohol Use Disorders Identification Test (AUDIT): WHO collaborative project on early detection of persons with harmful alcohol consumption. *Addiction*. 1993;88(6):791-804. PMID: 8329970

[2] Bush K, et al. The AUDIT alcohol consumption questions (AUDIT-C): an effective brief screening test for problem drinking. *Arch Intern Med*. 2003;163(7):821-829. PMID: 12695273

Section H ADHD

Based on ASRS-v1.1 | Kessler et al., Psychol Med 2005

Screens for adult ADHD including inattention and hyperactivity symptoms. Validated for adults aged 18 and above.

What it screens	ADHD in adults is often under-recognised. The Adult ADHD Self-Report Scale (ASRS-v1.1) was developed with the WHO and is the standard self-report ADHD screening tool for adults. It has two parts: Part A (6 questions) is the screener; Part B adds detail. This section uses the Part A items. A positive screen on 4 or more of the 6 Part A questions is consistent with adult ADHD. ADHD affects 2.5 to 4% of adults and is commonly missed, especially in women.
Scoring note	Standard 4-point scale (0 to 3). Total possible: 18. On the original ASRS, questions use a 5-point frequency scale with different cutoffs per question. For this app, use the 4-point scale and a total of 10 or above as the threshold for follow-up.

Domain Weights

Domain	Questions	Weight	Max Score
Inattention and Task Completion	ADH1, ADH2, ADH3, ADH4	65%	12
Hyperactivity and Restlessness	ADH5, ADH6	35%	6

Questions

For each question, tick the box that best describes how often you have experienced this over the past 2 weeks. RF column marks red flag questions.

#	Question	Not at all (0)	Several days (1)	More than half days (2)	Nearly every day (3)	RF
ADH 1	I have trouble wrapping up the final details of a project or task once the main challenge is done.	☐	☐	☐	☐	

ADH 2	I have difficulty getting things in order when I need to do a task that requires organisation or planning.	☐	☐	☐	☐	
ADH 3	I have problems remembering appointments or things I am supposed to do, even when I have written them down.	☐	☐	☐	☐	
ADH 4	I avoid or keep putting off starting tasks that require a lot of mental effort or sustained thinking.	☐	☐	☐	☐	
ADH 5	I fidget or squirm with my hands or feet when I have to sit down for a long time.	☐	☐	☐	☐	
ADH 6	I feel overly active and feel driven to do things, like there is a motor or engine inside me that I cannot switch off.	☐	☐	☐	☐	

No automatic red flag override for this section. Use total score and result tiers only.

References

[1] Kessler RC, et al. The World Health Organization Adult ADHD Self-Report Scale (ASRS): a short screening scale for use in the general population. Psychol Med. 2005;35(2):245-256. PMID: 15841682

[2] Adler LA, et al. Reliability and validity of self- and investigator ratings on the Adult ADHD Rating Scale (ADHD-RS). J Atten Disord. 2006;9(4):563-572. PMID: 16648213

Section I Eating Disorders

Based on SCOFF | Morgan et al.,
BMJ 1999

Screens for anorexia nervosa and bulimia nervosa. SCOFF is the standard eating disorder screen in primary care and psychiatry.

What it screens	Eating disorders include anorexia nervosa, bulimia nervosa, and binge eating disorder. The SCOFF questionnaire was developed at St George's Hospital, London, and is the most widely used brief eating disorder screen. On the original SCOFF, each Yes answer scores 1 and a total of 2 or more suggests an eating disorder. For this app it is adapted to the standard 4-point scale. Eating disorders have the highest mortality rate of any mental health condition and are frequently under-detected.
Scoring note	Standard 4-point scale (0 to 3). Total possible: 15. A score of 6 or above suggests clinically significant eating disorder symptoms. EAT1 and EAT3 are mandatory red flags at any score above 0.

Domain Weights

Domain	Questions	Weight	Max Score
Purging Behaviour	EAT1	25%	3 — Red Flag at any score
Loss of Control Over Eating	EAT2	20%	3
Significant Weight Loss	EAT3	25%	3 — Red Flag at any score
Body Image Distortion	EAT4	15%	3
Food Preoccupation	EAT5	15%	3

Questions

For each question, tick the box that best describes how often you have experienced this over the past 2 weeks. RF column marks red flag questions.

#	Question	Not at all (0)	Several days (1)	More than half days (2)	Nearly every day (3)	RF
EAT1	I make myself vomit because I feel uncomfortably full after eating.	☐	☐	☐	☐	!
EAT2	I worry that I have lost control over how much I eat.	☐	☐	☐	☐	
EAT3	I have lost more than 6 kg (about 13 pounds or one stone) in the last 3 months without trying to.	☐	☐	☐	☐	!
EAT4	I believe I am fat or overweight even when other people tell me I am too thin or underweight.	☐	☐	☐	☐	
EAT5	Food and thoughts about food dominate my life to the point where it affects my daily routine and relationships.	☐	☐	☐	☐	

Red Flag Rule: EAT1 (self-induced vomiting) and EAT3 (significant unintentional weight loss) are red flags. Any score of 1 or above on either question escalates to at least Moderate Concern. These symptoms are associated with serious physical health risks including electrolyte imbalance, cardiac arrhythmia, and malnutrition.

References

[1] Morgan JF, Reid F, Lacey JH. The SCOFF questionnaire: assessment of a new screening tool for eating disorders. BMJ. 1999;319(7223):1467-1468. PMID: 10582927

[2] Perry L, et al. Screening for symptoms of eating disorders in primary care. Int J Eat Disord. 2002;32(2):177-180. DOI: 10.1002/eat.10066

Section J Suicide Risk

Adapted from C-SSRS | Posner et al., Am J Psychiatry 2011

This section must always appear last. It is only shown if DEP9 scores 1 or above, OR if the user requests it. Every question is a red flag.

What it screens

The Columbia Suicide Severity Rating Scale (C-SSRS) is the global standard for assessing suicide risk and is used by the FDA, WHO, and military organisations worldwide. This self-report adapted version covers ideation intensity and past behaviour. The questions move from passive ideation (wishing to be dead) through active ideation to intent and past attempts. Any positive response to SUI3, SUI4, or SUI5 requires immediate crisis intervention, not a score calculation.

Scoring note

SPECIAL HANDLING. Do not display a total score for this section. Each question is individually assessed. SUI1 and SUI2 positive = Moderate Concern + provide helpline numbers. SUI3 or SUI4 positive = High Concern + immediate crisis resources. SUI5 positive = immediate crisis resources + clinical escalation note.

Domain Weights

Domain	Questions	Weight	Max Score
Passive Ideation	SUI1	—	Red Flag — Show crisis resources

Active Ideation Without Plan	SUI2	—	Red Flag — Show crisis resources
Active Ideation With Plan	SUI3	—	Red Flag — Immediate escalation
Intent to Act	SUI4	—	Red Flag — Immediate escalation
Past Attempts	SUI5	—	Red Flag — Immediate escalation + clinical note

Questions

For each question, tick the box that best describes how often you have experienced this over the past 2 weeks. RF column marks red flag questions.

IMPORTANT: Do not calculate or display a total score for this section. Each question is handled individually. Show crisis resources immediately alongside any positive response. In India: iCall 9152987821 | Vandrevala Foundation 1860-2662-345 | AASRA 9820466627

#	Question	Not at all (0)	Several days (1)	More than half days (2)	Nearly every day (3)	RF
SUI1	I have wished I were dead or that I could go to sleep and not wake up.	☐	☐	☐	☐	!
SUI2	I have had thoughts of killing or harming myself.	☐	☐	☐	☐	!
SUI3	I have thought about how I might do it, including what method I would use.	☐	☐	☐	☐	!
SUI4	I have had a strong intention or urge to act on thoughts of harming or ending my life.	☐	☐	☐	☐	!
SUI5	I have done something to try to harm or end my life in the past.	☐	☐	☐	☐	!

Red Flag Rule: Every question in this section is a red flag. SUI1 or SUI2 scoring above 0: show crisis helpline numbers immediately. SUI3 or SUI4 scoring above 0: interrupt the flow and display a full crisis resource screen. SUI5 scoring above 0: add a clinical escalation note in the report alongside crisis resources. In India: iCall helpline 9152987821. Vandrevala Foundation 1860-2662-345. AASRA 9820466627.

References

- [1] Posner K, et al. The Columbia-Suicide Severity Rating Scale: initial validity and internal consistency findings from three multisite studies with adolescents and adults. Am J Psychiatry. 2011;168(12):1266-1277. PMID: 22193671
- [2] Mundt JC, et al. Prediction of suicidal behavior in clinical research by lifetime suicidal ideation and behavior ascertained by the C-SSRS. J Psychiatr Res. 2013;47(11):1666-1674. PMID: 23948581

K Summary Table and Key References

Section	Condition	Questions	Scale	Max Score	Clinical Basis	Red Flags
A	Depression	9	0 to 3	27	PHQ-9	DEP9

B	Generalised Anxiety	7	0 to 3	21	GAD-7	None
C	Bipolar Disorder	7	0 to 3	21	MDQ	BIP6
D	PTSD	8	0 to 3	24	PCL-5	PTS8
E	OCD	6	0 to 3	18	OCI-R	None
F	Psychosis	6	0 to 3	18	PRIME Screen	PSY1 PSY2 PSY3 PSY4 PSY6
G	Alcohol Use	5	Mixed 0-4	20	AUDIT	ALC3 ALC5
H	ADHD	6	0 to 3	18	ASRS-v1.1	None
I	Eating Disorders	5	0 to 3	15	SCOFF	EAT1 EAT3
J	Suicide Risk	5	Special	—	C-SSRS	All 5 questions
	TOTAL	64				

Key Clinical Resources

References
PHQ-9: Kroenke K, Spitzer RL, Williams JB. J Gen Intern Med. 2001;16(9):606-613. PMID: 11556941
GAD-7: Spitzer RL, et al. Arch Intern Med. 2006;166(10):1092-1097. PMID: 16717171
MDQ: Hirschfeld RM, et al. Am J Psychiatry. 2000;157(11):1873-1875. PMID: 11058490
PCL-5: Weathers FW, et al. National Center for PTSD, VA. 2013. www.ptsd.va.gov
OCI-R: Foa EB, et al. Psychol Assess. 2002;14(4):485-496. PMID: 12501574
PRIME Screen: Miller TJ, et al. Schizophr Bull. 2003;29(4):703-715. PMID: 14989408
AUDIT: Saunders JB, et al. Addiction. 1993;88(6):791-804. PMID: 8329970
ASRS-v1.1: Kessler RC, et al. Psychol Med. 2005;35(2):245-256. PMID: 15841682
SCOFF: Morgan JF, Reid F, Lacey JH. BMJ. 1999;319(7223):1467-1468. PMID: 10582927
C-SSRS: Posner K, et al. Am J Psychiatry. 2011;168(12):1266-1277. PMID: 22193671
DSM-5: American Psychiatric Association. Diagnostic and Statistical Manual of Mental Disorders, 5th ed. 2013.
ICD-11: World Health Organisation. International Classification of Diseases, 11th Revision. 2022. https://icd.who.int
India Crisis Resources: iCall 9152987821 Vandrevala Foundation 1860-2662-345 AASRA 9820466627

Final Note: This framework is for informational and development purposes only. The composite scoring model described in this document has not been independently validated in a clinical trial. Before deploying any version of this application publicly, independent clinical validation, ethics review, and legal review are required. For clinical enquiries: DSM-5 (APA, 2013) and ICD-11 (WHO, 2022) are the diagnostic standards used in India and globally.